

1. Administrative & Study Identification

Full Study Title: An open-label phase 1 study to investigate SGN-CEACAM5C in adults with advanced solid tumors **Short Title/**
Acronym: SGNCEA5C-001 **Protocol Number:** SGNCEA5C-001 **NCT Number:** NCT06131840 **Trial Status:** Recruiting **Sponsor Name:** Pfizer
Investigational Product: PF-08046050 (SAR445953; SGN-CEACAM5C) - an antibody-drug conjugate (ADC) targeting CEACAM5 with a topoisomerase 1 inhibitor payload (AMDCPT) **Phase of Development:** Phase 1 **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To characterize the safety, tolerability, pharmacokinetics (PK), pharmacodynamics (PD), and preliminary antitumor activity of PF-08046050 (SGN-CEACAM5C) in adults with select advanced solid tumors. **Secondary Objectives:** To evaluate Pharmacokinetic parameters, Objective Response Rate (ORR), Duration of Response (DoR), Progression-Free Survival (PFS), and Overall Survival (OS).

2.2 Background & Rationale Patients with recurrent or metastatic solid tumors—including non-small cell lung cancer (NSCLC), small cell lung cancer (SCLC), colorectal cancer (CRC), gastric adenocarcinoma (GC), and pancreatic ductal adenocarcinoma (PDAC)—have limited treatment options. Carcinoembryonic antigen-related cell adhesion molecule 5 (CEACAM5) is highly expressed across these tumors. SGN-CEACAM5C is a novel investigational ADC directed to CEACAM5 that delivers a topoisomerase 1 inhibitor payload to selectively kill cancer cells, addressing a critical unmet medical need for patients who have progressed on standard therapies.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Interventional **Control Type:** Single-arm (Single Group Assignment) **Blinding:** Open-label
Randomization: N/A

3.2 Planned Enrollment & Duration **Target \$N\$:** 494 participants
Number of Sites: Multicenter (Global, including sites in the US, Canada, and Europe) **Estimated Study Start:** Late 2023 **Estimated Primary Completion:** Approximately 2025-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18 years with histologically- or cytologically-confirmed metastatic or unresectable solid tumor malignancy/adenocarcinoma (CRC, GC, GEJ, NSCLC, PDAC, or SCLC). Part D and E specifically require unresectable or metastatic adenocarcinoma of the colon or rectum. 2. Relapsed, refractory, or progressive disease with no appropriate standard therapy available at the time of enrollment. Participants in expansion and combination cohorts (Parts C, D, E) must have met specific prior lines of therapy requirements (e.g., prior fluoropyrimidine, oxaliplatin, irinotecan, anti-VEGF, or anti-EGFR depending on tumor type and cohort). 3. Measurable disease per RECIST v1.1 and an Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 4. Participants in Part B (monotherapy dose optimization), Part C (monotherapy dose expansion), and Part E (bevacizumab combination dose expansion) must have a tumor site accessible for biopsy and agree to biopsy(ies) and/or submission of archival tissue.

4.2 Exclusion Criteria 1. Previous exposure to CEACAM5-targeted therapy or an ADC with a camptothecin payload. 2. History of another malignancy within 3 years before the first dose of study intervention, or any evidence of residual disease from a previously diagnosed malignancy. 3. Active cerebral/meningeal disease related to the underlying malignancy (participants with treated and clinically stable CNS disease are allowed if off steroids for at least 2 weeks with no ongoing related AEs). 4. For participants in bevacizumab combination cohorts (Parts D and E): History of allergic reactions or hypersensitivity to bevacizumab, its excipients, Chinese Hamster Ovary (CHO) cell products, or other recombinant human/humanized antibodies. 5. For participants in bevacizumab combination cohorts (Parts D and E): Serious non-healing wound, ulcer, or bone fracture; deep venous thromboembolic event within 4 weeks prior to enrollment; known coagulopathy or bleeding diatheses; history of any life-threatening VEGF-related adverse event.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: PF-08046050 (SGN-CEACAM5C) is administered in specific cycles. The study has 5 parts: monotherapy dose escalation (Part A), monotherapy dose optimization (Part B), monotherapy dose expansion (Part C), bevacizumab combination therapy dose escalation (Part D), and bevacizumab combination therapy dose expansion (Part E). **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Until disease progression, unacceptable toxicity, or study withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care and combinations explicitly evaluated in the

trial cohorts (e.g., bevacizumab [Avastin, Vegzelma, Alymsys, Zirabev, Mvasi] for Parts D and E). **Prohibited:** Prior TOP01 inhibitor (such as irinotecan or nanoliposomal irinotecan) in any setting for specific combination cohorts, unless permitted by prior line inclusion requirements for expansion parts.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Tumor Imaging (CT/MRI), Archival tissue/Biopsy submission.
Baseline	Day 0	First Dose of PF-08046050, Baseline PK/PD blood draws.
Interim	Every Cycle	Safety monitoring, AE evaluation, PK/PD assessment, and periodic radiographic tumor evaluations per RECIST v1.1.
End of Study	Follow-up	Final safety assessments (30-37 days after the last dose) and long-term survival tracking (up to ~2 years).

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints 1. Number of participants with adverse events (AEs). 2. Number of participants with laboratory abnormalities. 3. Number of dose modifications due to AEs.

7.2 Secondary & Exploratory Endpoints 1. Pharmacokinetic (PK) parameter - Area under the concentration-time curve (AUC). 2. PK parameter - Maximum concentration (Cmax). 3. PK parameter - Time to maximum concentration (Tmax). 4. Progression-free survival (PFS) based on radiographic assessments per RECIST v1.1. 5. Objective Response Rate (ORR), Duration of Response (DoR), and Overall Survival (OS).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous collection from informed consent through 30-37 days post-treatment. **Serious Adverse Events (SAEs):** Expedited reporting within 24 hours to the sponsor. **Data Monitoring Committee (DMC):** Internal Safety Review Committee assessing Dose Limiting Toxicities (DLTs) to guide the dose escalation decisions for Parts A, B, and D.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients receiving ≥ 1 dose) for safety assessments; Efficacy Evaluable Set for tumor responses. **Sample Size Justification:** Sample size of 494 is driven by standard Phase 1 adaptive dose-escalation/optimization

design to identify the Maximum Tolerated Dose (MTD) and Recommended Phase 2 Dose, followed by expansion cohorts. **Handling of Missing Data:** Efficacy endpoints involving time-to-event outcomes (PFS and OS) will utilize Kaplan-Meier methods, censoring missing data or patients lost to follow-up appropriately.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular clinical site monitoring (onsite and remote) to verify source data, protocol adherence, and GCP compliance. **Audit Trail:** Validated Electronic Data Capture (EDC) system with full audit trails and data clarification mechanisms.

11. Study Identifiers & Governance

Primary ID: SGNCEA5C-001 **Registry Number:** NCT06131840 **Posting ID:** 331839925337539851 **Sponsor/Lead Organization:** Pfizer **Study Title:** A Study of SGN-CEACAM5C in Adults With Advanced Solid Tumors
Official Regulatory Title: An Open-label Phase 1 Study to Investigate SGN-CEACAM5C in Adults With Advanced Solid Tumors

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced Solid Tumors, Adenocarcinoma **Disease Areas:** Colorectal Neoplasms, Carcinoma, Non-Small-Cell Lung, Stomach Neoplasms, Pancreatic Ductal Adenocarcinoma, Gastroesophageal Junction Adenocarcinoma, Small Cell Lung Carcinoma **Diagnosis Threshold:** Histologically- or cytologically-confirmed metastatic or unresectable malignancy with measurable disease **Medical Methodology:** Targeted Antibody-Drug Conjugate (ADC) therapy **Optimization Target:** Establishing the safest and most effective dose (MTD and RP2D)

13. Study Procedures & Methodology

Management Pathway: Clinical evaluation via RECIST v1.1 and comprehensive safety tracking **Education Protocol (HCP):**

Investigator site training on ADC toxicities and dose-modification guidelines **Education Protocol (Patient):**

Comprehensive informed consent process detailing the experimental nature of Phase 1 ADC therapy and necessary biopsy requirements

Quality Audit Mechanism: Central review of imaging and sponsor-led clinical quality assurance audits

14. Observation & Follow-Up Schedule

Total Duration: Up to approximately 2 years **Onsite Visit Cadence:** Coincides with treatment cycles and defined imaging intervals (e.g., every 3-4 weeks) **Remote Monitoring Frequency:** Continuous pharmacovigilance and survival follow-up calls after treatment discontinuation **Checklist Review Interval:** Routine safety laboratory reviews prior to each dosing administration

15. Sign-off & Approval

Role	Name	Signature	Date						
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]						
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]						
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]						